

المجلس الطبي المتخصص

Name: حسنيه محمد محمد عظم ط
National ID: 26209302600389



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Mammography and breast ultrasound

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

حسنية
محمد محمد عطوط

٢٥ ش ٢ - عرب راشد الشرقيه
حلوان - القاهره

١٩٦٧/٠٩ ٢٦٢٠٩٣٠٢٦٠٠٣٨٩

LW1165321

٢٠٢٦/٠٧ ٢٦٢٠٩٣٠٢٦٠٠٣٨٩

ربة منزل

أنثى
مصلحة
على حسن على الصعيدي

البطاقة سارية حتى ٢٠٣٣/٠٧/١٥

متزوجة

Patient Name	: الأستاذة / حسنية محمد محمد عطوط	Request No.	: 13007417
Patient No.	: 26209302600389	Request Date	: 24/05/2025
Path No.	: 1890/25	Verification Date	: 11/06/2025
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 62 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data

Known case of right breast cancer with enlarged right axillary LN

Nature of specimen

Tru cut biopsy , collected at 22/5/2025 , 1:00 pm

Gross examination

Multiple grayish white tissue cores 1.5 cm , totally submitted.

Microscopic examination

Microscopic examination revealed normal nodal tissue with patent subcapsular sinuses .
The lymphoid follicles are hyperactive and have prominent germinal centers. There is no evidence of specific granuloma or neoplasia in sections examined.

Diagnosis

Right axillary LN; Tru cut biopsy :

Reactive lymph node .

No metastatic deposits .

Signature

Dr. Rana Fathy

Dr.Heba Hassan

Verified by :

Printing Date : 12/06/2025 14:29:40

Page 1 of 1

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Hosneya Mohamed Mohamed Ataot	Date : 30-04-2024
Exam : C.T ABDOMEN & PELVIS	Age : 61 Year(s)
Medical ID: 912/2024	49676

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Status post right mastectomy showing operative bed distortion and scarring as well as prominent right axillary lymph node measuring 1.2 X 1 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- Left basal atelectatic band.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Right thyroid lobe calcified nodule.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : حسنية محمد محمد عطوط

Age : 61 Y : 3 M : 2 D

Referred By Prof. Dr. : عصمت الجابري

Clinical Diagnosis : Cancer breast

Nature of Specimen : MRM

Receiving Date : 2024-01-26

Analysis ID : 821630

Gender : Female

Patient ID : 622172

PATHOLOGY REPORT

Gross :

Specimen received consists of MRM with axillary clearance measured 23x18x5 cm, covered by skin ellipse measured 20x9 cm.

Nipple and areola are grossly unremarkable.

Cut section revealed mass with firm gritty cut section and ill defined borders, measured 3x2x1.5 cm, located at 0.5 cm away from the least margin (deep). Rest of cut section showed multiple ill defined rubbery tan areas with cystifications measured up to 6x6 cm. All surgical margins are free.

Dissection of the axilla revealed 23 lymph nodes, the largest measured 2.8x1.5x1 cm.

Microscopic :

Sections examined from the specimen received revealed breast tissue showing tumour tissue formed of irregular groups of malignant ductal cells showing moderate nuclear pleomorphism, hyperchromasia.

There is moderate attempt at tubular formation.

The mitotic index is low.

There is marked desmoplasia.

There is peritumoural lymphocytic infiltrate around 5%.

Lympho-vascular space invasion absent.

Perineural invasion absent.

There is an intraductal component about 5% formed of dilated ducts with malignant lining showing solid and papillary patterns.

The surrounding breast tissue showed fibrocystic change, usual ductal hyperplasia and intraduct papilloma.

Sections from the nipple and areola are free.

Surgical margins are free.

Sections examined from the axillary nodes revealed no metastatic deposits in 23 nodes.



عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

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^{99m}Tc MDP Bone Scan Report (25/5/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP Revealed:

- Heterogeneous tracer uptake is seen over dorso-lumbar vertebrae?? Pronounced degenerative changes?? Otherwise.
- No definite evidence of active osseous deposits could be seen over the rest of the surveyed skeleton.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- Non-uniform tracer uptake over mid and lower dorsal as well as lumbar vertebrae, degenerative nature is more favored than bone deposits for further clinical, CT cuts correlation and serial scans.
- No definite evidence of active metastatic osseous deposits could be seen all over the rest of the surveyed skeleton.

Much Obligated

Alaa Elsaeed

Mohamed Abdeltawab, MSc

Prof. Ahmed Zaher, MD